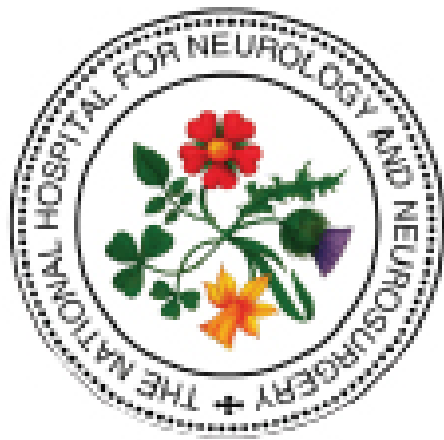




The National Hospital for Neurology and Neurosurgery Neuroscience Critical Care Unit (NCCU)

PRE-PROCEDURE FASTING

These guidelines are to be followed for patients undergoing a procedure that requires fasting or interruption of NG feeding

Pre-operative / pre-procedure – includes operations and procedures in theatre, in NCCU and other departments such as radiology where sedation or general anaesthesia is necessary

FOOD / MILK

Includes solid food, soup, milk, milky tea, coffee, sweets, NG / PEG feed

WATER / CLEAR FLUID

- Patients should drink clear fluid freely up until 2 hours before procedure
- If patient does not like water, other clear fluids are allowed - black tea, diluted juice squash and other non-fizzy clear drinks (*clear = newsprint should be visible through the glass of liquid*)
- Non-carbonated rehydration drinks are good source of carbohydrate and electrolytes and patients without diabetes having elective procedures should be encouraged to drink these
- Recommended maximum water intake from 6 to 2 hours pre-operatively is 1000mL for adults

ORAL MEDICATION

- Being nil by mouth is not a reason for patients not to take oral medication
- Oral medication may be taken during the fasting period with sips of water
- Medications should be continued preoperatively following discussion with medical staff

RESTARTING NG FEED

Reconfirm NG tube position prior to restarting NG feed in all cases

Restart at pre-procedure rate

- Immediately on return** after non-abdominal surgery, scans, interventional neuroradiology with a protected airway
- 4 hours** after airway procedures such as tracheostomy or extubation if stable

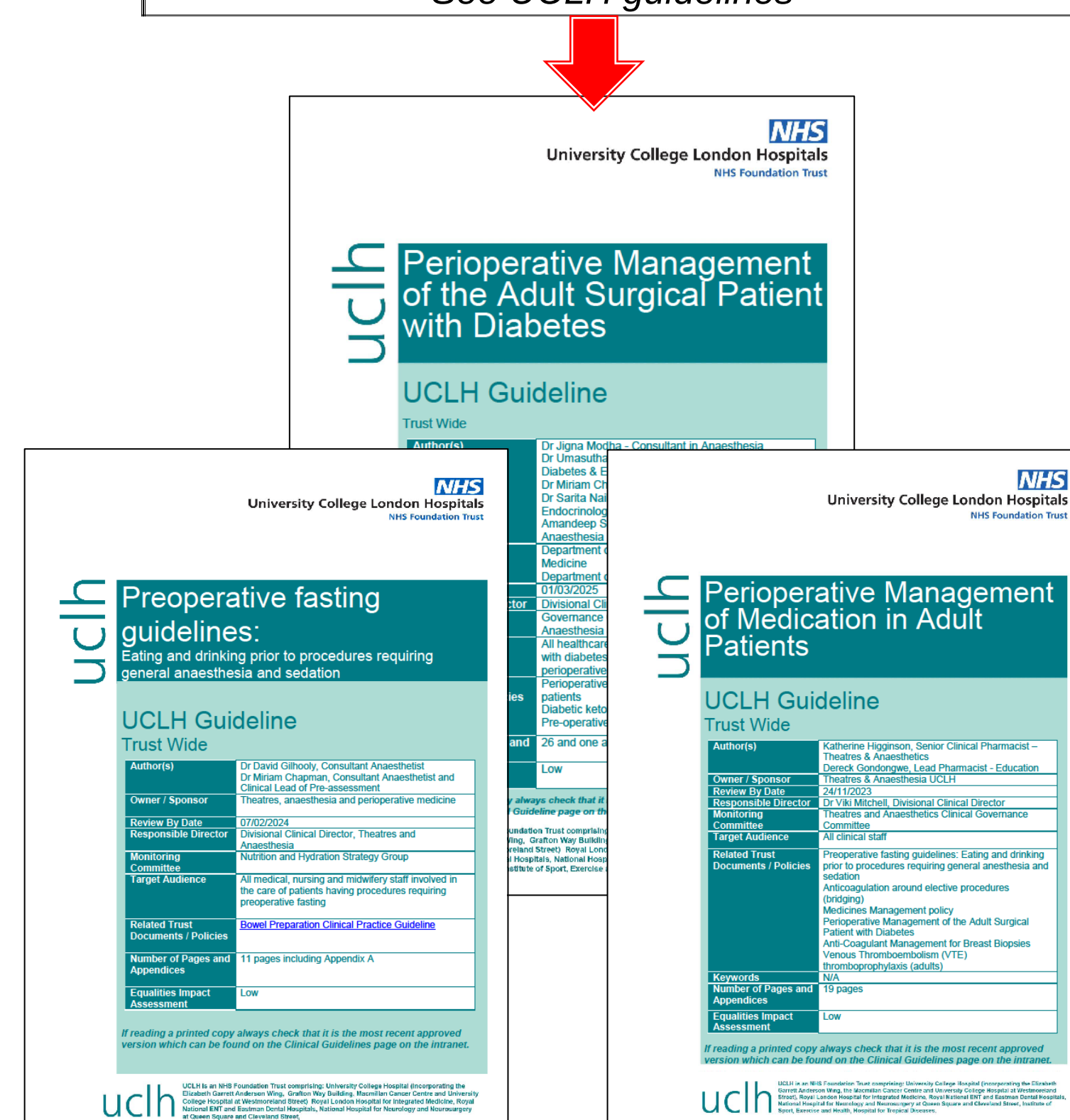
FASTING PRIOR TO PLANNED SURGERY / INTERVENTION NON-INTUBATED PATIENTS

- No food or milk for **6 hours** pre-operatively
- No chewing gum for **2 hours** pre-operatively
- No water or clear fluids for **2 hours** pre-operatively
- Essential medications taken as agreed with medical staff

PATIENTS ON ORAL HYPOGLYCAEMICS / INSULIN

Anaesthesia and surgery can lead to complex metabolic and neuroendocrine responses and perioperative management of diabetic patients requires careful assessment, planning and co-ordination

See UCLH guidelines



STOPPING ENTERAL NUTRITION PRIOR TO PLANNED SURGERY / INTERVENTION

INTUBATED PATIENTS and PATIENTS with TRACHEOSTOMY

BEFORE NON-AIRWAY SURGERY (Neurosurgery) / INTERVENTIONAL NEURORADIOLOGY

- NO FASTING** – enteral feed continued until preparing patient for procedure
- NGT ASPIRATED**, gastric contents discarded and tube capped prior to leaving Neurocritical Care Unit (NCCU)

BEFORE AIRWAY PROCEDURES

Percutaneous or Surgical Tracheostomy / Airway Change / Extubation / Bronchoscopy

- Time for procedure should be confirmed
- Enteral feed stopped **4 hours** prior to procedure
- If patient to be on emergency list, fast from **0400** as timing for procedure may be subject to change
- NGT ASPIRATED**, gastric contents discarded and tube capped prior to leaving NCCU

BEFORE CT SCAN / MRI

- NO FASTING** – enteral feed continued until preparing patient to leave NCCU – exception *PET scan which requires 6 hours fasting to ensure empty stomach*
- NGT ASPIRATED**, gastric contents discarded and tube capped prior to leaving NCCU

BEFORE TRANSOESOPHAGEAL ECHO (TOE)

- Time for procedure should be confirmed
- Enteral feed stopped **4 hours** prior to procedure
- NGT ASPIRATED** and gastric contents discarded immediately prior to procedure